SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

CAP: advanced salivary gland cancer

Advanced salivary gland cancer · NCCP 00615 · version 2 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

Schedule
Doxorubicin 50 mg/m², cisplatin 50 mg/m² and cyclophosphamide 500 mg/m² on Day 1, repeated every 21 days for up to 6 cycles, stopping earlier for progression or unacceptable side effects.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Cyclophosphamide
CYTOTOXIC CHEMOTHERAPY

- ☐ Low blood counts, infection, bleeding and anaemia
- ☐ Sickness, mouth ulcers, diarrhoea, tiredness and hair loss
- ☐ Cyclophosphamide-related bladder irritation and blood in urine
- ☐ Fertility effects, pregnancy risk and rare later second cancers

Doxorubicin
CYTOTOXIC CHEMOTHERAPY

- ☐ Low blood counts, infection, bleeding and anaemia
- ☐ Sickness, mouth ulcers, diarrhoea, tiredness and hair loss
- ☐ Doxorubicin-related heart damage and tissue injury if it leaks outside a vein
- ☐ Fertility effects, pregnancy risk and rare later second cancers

Cisplatin
CYTOTOXIC CHEMOTHERAPY

- ☐ Low blood counts, infection, bleeding and anaemia
- ☐ Sickness, mouth ulcers, diarrhoea, tiredness and hair loss
- ☐ Cisplatin-related kidney damage, low blood salts, hearing loss and nerve damage
- ☐ Fertility effects, pregnancy risk and rare later second cancers

Discuss treatment aim, expected benefit and uncertainty, alternatives and the option of declining; each medicine's risks, potentially lasting effects, required tests, urgent symptoms, fertility, contraception, pregnancy, breastfeeding and the individual support plan. Follow the unit's cisplatin hydration plan and individual fluid target. The source advises 2–3 litres on Day 1 to reduce bladder irritation from cyclophosphamide; the team must adapt this if heart or kidney problems restrict fluids. Report blood in urine or painful urination promptly.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00615 v2.

SACTCheck v0.84.50
Developed by Dr Paul O'Brien